BETAHISTINE

Ménière's disease

Take with food

CHEMICAL NAME N-methyl-2-(pyridin-2-yl)ethanamine (dihydrochloride or mesilate)
CLASS Histamine analogue (H1 weak agonist / H3 antagonist) antivertigo agent
TRADE NAMES Serc · Betaserc · Vertin · Betavert

THERAPEUTIC USES / INDICATIONS

- Ménière's disease: vertigo, tinnitus, hearing loss.
- Vestibular vertigo (off-label in many settings).

PHARMACODYNAMICS

- Weak **H1 agonist** and strong **H3 antagonist** in inner ear and brainstem → ↑ cochlear and vestibular blood flow, ↓ endolymph pressure.
- Facilitates vestibular compensation.

ADMINISTRATION

- **PO** with food (reduces GI upset).
- Benefit develops over weeks.

SIDE & ADVERSE EFFECTS

SIDE EFFECTS (COMMON)

- Nausea, dyspepsia.
- Headache.
- Bloating.

ADVERSE EFFECTS (SERIOUS)

- Hypersensitivity, skin rash, pruritus.
- Bronchospasm in asthmatics (rare).

DOSAGES

- **Adult:** 16 mg three times daily initially; maintenance 24–48 mg/day in divided doses.
- Not recommended in children.

CONTRAINDICATIONS & CAUTIONS

- Phaeochromocytoma.
- **Caution:** asthma, peptic ulcer history, children, pregnancy.

KEY INTERACTIONS

- **Antihistamines:** may reduce effect.
- **MAOIs:** ↑ levels.

PHARMACOKINETICS

- **Peak:** 1 h. Rapid absorption.
- Metabolised (MAO) to 2-pyridylacetic acid (inactive).
- Excreted in urine. **t½:** 3–4 h.

NURSING MANAGEMENT

- Assess frequency and severity of vertigo attacks.
- Fall precautions.
- Monitor asthmatics for wheeze.

PATIENT EDUCATION

- Take with food; benefit may take several weeks.
- Low-salt diet and limiting caffeine may help Ménière's.
- Report rash or wheezing.

KEY POINTS

- H3 antagonist / weak H1 agonist.
- Avoid in phaeochromocytoma.
- Ménière's: 16 mg TDS.
- Caution in asthma and ulcer.

SOURCES CHECKED **[544]** Betahistine 16 mg tablets (UK SmPC)

Full references p. 432–457